

SR-002: Arthur Kovacs — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	SR-002
Persona	Senior Resident	Next Step	Feed into `/summarize-interview` with other SR interviews

SESSION DETAILS

Date	2026-04-03	Duration	52 min
Interviewer	PM Lead	Setting	Face-to-face, slow pace
Location	Arthur's apartment (small, tidy, photos of wife Elena everywhere)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your daily routine --- what does a typical day look like?

I wake up around seven. Elena always woke up first and made coffee. Now I make it myself. It takes longer because my hands aren't as steady. The nurse comes Monday and Thursday. The care worker comes Monday, Wednesday, and Friday for bathing and tidying. Michael — my son — comes on Sundays. Meals on Wheels delivers lunch on weekdays. The rest of the time\... I'm here. I watch the cricket if it's on. I read engineering journals sometimes. I sit.

[Probe] How long has it been since Elena passed?

Eighteen months. She had pancreatic cancer. Six weeks from diagnosis to\... [pause] She managed everything. The house, the bills, my medications, our social life. When she died, I didn't just lose my wife. I lost my whole system. She used to lay out my pills every morning in a little cup on the counter at seven o'clock. Now I have the organiser the nurse fills. But I can't always remember what day it is, especially on weekends when nobody comes. All the days feel the same.

[Emotional note] Arthur's eyes filled but he didn't cry. He touched the photo frame on the table next to him. The grief is integrated, not raw, but present in everything.

Block 1: Falls (adapted for SR)

Q: Can you tell me about the falls you've had?

Twice, both in the bathroom. The first time, about eight months ago, I slipped getting out of the bath. I grabbed the rail and managed to get up after about twenty minutes on the floor. Didn't tell anyone. It was embarrassing more than anything. I just thought, 'I need to be more careful.' The second time was four months ago. Same thing — getting out of the bath. But this time I couldn't get up. I pressed the emergency button on the wall — the agency installed it. And I waited. On the wet floor. Cold. For thirty-five minutes. Wondering if the button actually worked or if I was just going to lie there until someone noticed I was missing.

[Probe] What was going through your mind during those thirty-five minutes?

Frank. My neighbour across the hall. He died eight months ago. His daughter found him on a Tuesday. The neighbours said they hadn't seen him since Saturday. Three days. That's what I was thinking about on the

bathroom floor. Whether I was going to be Frank. I sleep with the phone on my pillow now. And the hallway light stays on all night. I'm not afraid of dying. I'm afraid of dying and nobody finding me for three days.

[Emotional note] Steady voice but his hand trembled on the table. This fear is present every night.

I don't bathe on days when nobody's scheduled to come. If I fall on a Saturday, the next visit isn't until Monday morning. Thirty-six hours. So I only bathe when the care worker is coming.

Block 2: Medications (adapted for SR)

Q: How do you manage your medications?

The nurse fills the organiser. Elena used to put the pills in a cup — just the morning ones, right there on the counter when I came to the kitchen. I never missed a dose when she was here. Now I open the organiser and sometimes I can't remember if today is Wednesday or Thursday. On weekdays it's easier because someone usually comes — the nurse, the care worker, Meals on Wheels. That anchors the day. Weekends are the worst. Nobody comes. Every day feels like every other day.

Q: Have you ever taken a dose twice or skipped one?

Both, probably. I try to be careful but I can't always be sure. My son Michael set an alarm on the tablet he gave me but I don't always hear it. And sometimes I hear it and I think, 'Did I already take them, or is this the alarm telling me to take them?' It's confusing. I don't tell Michael because he worries enough as it is. He works hard. He has his own problems.

Block 3: Communication & Information (adapted for SR)

Q: When you notice something about your health, what do you do?

I told the nurse last month that my ankles seemed more swollen than usual. She wrote it down. Two weeks later, my doctor mentioned it at the regular visit as if it was new information. So what was the point of me saying anything? If the information I give the nurse doesn't reach the doctor, then I'm talking into a void. I might as well tell the wall.

[Probe] How did that make you feel?

Like I don't matter in my own care. The nurse writes it down, the note sits in a system somewhere, and the doctor sees whatever the doctor sees. I don't know what my care plan says. Nobody has ever shown it to me. I assume there's a plan because people keep showing up, but what are they looking for? What am I supposed to be doing? I'd like to know.

Block 3b: Trust in Technology (adapted for SR --- Amendment A3)

Q: Has technology ever been used to help monitor your health?

Yes, and it went badly. My previous agency — about eight months ago — they had some kind of AI system that listened to phone calls. 'Wellness monitoring,' they called it. After two weeks, my son got a call saying the system detected signs of depression in my voice. The system. Not a person. A computer listened to how I talked on the phone and decided I was depressed. I wasn't depressed. I was exhausted. There was construction noise upstairs — three nights of jackhammering. Of course I sounded flat. Of course I was speaking slowly. But did anyone call to ask, 'Arthur, you sound tired, everything okay?' No. They went straight from 'computer says he's depressed' to 'tell his son and his GP.' Michael called me in a panic. 'Dad, are you okay? The system says you might be depressed.' I was furious. Absolutely furious. A machine told my son I was mentally ill before it told me. Before anyone asked me. The GP discussed it at the next appointment. She wrote 'AI flag reviewed and clinically dismissed.' But it's still there in my record. Every new nurse who opens my file sees 'Mental Health: AI-flagged depressive indicators.' What do you think they see first — the flag or the dismissal?

[Emotional note] Arthur's hand was flat on the table, pressing down. Controlled anger, not frustration --- this felt like a violation to him.

Now when the nurse calls me for a check-in, I make sure I sound cheerful. I speak clearly. I say I'm fine even when I'm not. Because if I sound tired again, who knows what the computer will decide about me this time.

[Probe] So the monitoring changed how you communicate?

It did. I perform wellness now. That's what I call it. I perform for the phone. And the worst part is — the system was supposed to detect when something's wrong, but now I'll never let it detect anything. So it's actually made things worse. If I am depressed one day, truly depressed, they won't know, because I'll sound exactly the same as every other day. They taught me to hide.

Block 4: Care Worker Continuity (adapted for SR)

Q: How many different care workers have visited you recently?

Three different people in the last two months. My regular is Lisa. She knows where everything is, she knows I don't like the bathroom rearranged, she knows I take my coffee black with one sugar. When Lisa doesn't come, someone else comes. Last time, the replacement moved my soap and my towel to different hooks. Elena put those there. I didn't say anything but I moved them back after she left. Nobody briefs them about me. They come in, they do the checklist, they leave. Lisa sits down for five minutes and asks about the cricket. The others do the job. There's a difference between being cared for and being processed.

Block 5: Willingness (adapted for SR)

Q: If there were something that helped your care team know you better, would you be open to that?

Depends what it is. If it's another machine that listens to me and makes decisions about my mental health — absolutely not. They call it 'wellbeing monitoring.' I call it a machine that listens to how you talk and then tells people you're sick. There's a word for that. It's not 'care.' But if it meant that when a new care worker comes, they already know: Arthur takes his coffee black, don't move the bathroom things, ask about the cricket, and by the way he's a former mechanical engineer so don't talk to him like he's a child — that would be good. That would be care.

Wrap-Up

Q: Is there anything I didn't ask that's important?

Ask about loneliness. Nobody does. The care workers come and go. Michael comes on Sundays. The rest of the time, it's me and Elena's photos. I'm not asking for company — I know people are busy. But when the nurse comes and spends the entire visit on her phone, that half hour was supposed to be human contact. And instead it was someone checking boxes in my living room while I watched. Whatever you build, make it so the people who come to my home can look at me when they talk to me. That's all I'm asking. Give them whatever they need before they walk through my door so they don't have to spend my visit typing. Give me that half hour back as a human being, not a data point.

PART B — Filled Note Template

Participant ID	SR-002
Persona	SR
Date	2026-04-03
Duration (min)	52
Interviewer(s)	PM Lead

KEY JOBS	Survive safely in his apartment. Not be forgotten. Maintain connection to Elena's memory. Be treated as a person, not a checklist. Get through weekends alone. Not become Frank.
CURRENT SOLUTION	Emergency button (response: 35 min). Pill organiser (confusion on weekends). Meals on Wheels (weekday anchor). Care workers 3x/week. Son on Sundays. Phone on pillow. Hallway light on all night. Only bathes on visit days. "Some computer decided I was depressed because I was talking slowly. I wasn't depressed. I was tired. But they told my son and my doctor before they told me." "I sleep with the phone on my pillow and the hallway light on. I'm not afraid of dying. I'm afraid of dying and nobody finding me for three days." "I perform wellness now. If I am depressed one day, truly depressed, they won't know." "When a new care worker comes, they should already know: Arthur takes his coffee black, don't move the bathroom things, ask about the cricket." "Give them whatever they need before they walk through my door so they don't have to spend my visit typing. Give me that half hour back as a human being."
WILLINGNESS	Open to systems that help care workers know him better. Open to seeing own care plan. NOT open to any voice/mood monitoring. NOT open to systems that notify family before him. Key: technology should increase human presence during visits, not replace it.
EMOTIONAL INTENSITY	Eyes filled but didn't cry when discussing Elena. Hand trembled describing Frank. Controlled anger about AI depression flag ('violation'). Grief-integrated loneliness throughout. Final request about human connection was the emotional peak.
ESCALATION BEHAVIOR	Doesn't escalate. Doesn't tell son about missed appointments, dizziness, eating problems ('he has his own problems'). Reported swollen ankles once — info was lost — concluded reporting is pointless. Now performs wellness on phone calls. Avoids bathing on non-visit days (risk avoidance, not risk reporting).
SURPRISE FINDING	PERFORMS WELLNESS: Arthur deliberately sounds cheerful on check-in calls because a previous AI system falsely flagged him as depressed. AI monitoring degraded data quality. Also: 'Frank story' — neighbour found dead 3 days later is the emotional anchor. Also: reported data (swollen ankles) was lost between nurse and GP. Also: care plan is invisible to the patient it's written for.
TOOL MENTIONS	Emergency wall button (35 min response). Pill organiser. Tablet (unused, gifted by son). Previous AI wellness monitoring system (negative experience). Phone (on pillow).
REFERRAL	Lisa (regular care worker — 'she's the only one who asks about the cricket'). Michael (son — 'but he'll tell you I'm fine because he needs to believe that').
FOLLOW-UP ACTIONS	[] Design patient-visible care plan in accessible language [] Care worker briefing system with patient preferences + sensitivities [] Investigate 'performing wellness' as data quality degradation risk [] Ensure all health inferences pass L2 clinical review before ANY notification [] Weekend isolation as medication drift risk factor